

MEDICAL REPORT

Hospital: CityCare Multispeciality Hospital

Department: General Medicine

Report Date: 20 August 2026

Report ID: MR-2026-00871

1. Patient Information

Patient Name: Rahul Sharma

Age: 42 years

Gender: Male

Patient ID: P-10452

Date of Visit: 18 August 2026

2. Chief Complaints

The patient presented with:

- Persistent fatigue for approximately 3 weeks
- Increased thirst and frequent urination
- Mild dizziness, particularly in the morning
- Occasional blurred vision
- Increased hunger despite regular meals

3. Medical History

The patient has a family history of Type 2 Diabetes Mellitus. He reports a sedentary lifestyle and irregular dietary habits.

No previous history of major cardiovascular disease was reported.

The patient does not report any known drug allergies.

4. Physical Examination

Blood Pressure: 138/88 mmHg

Heart Rate: 82 beats/min

Respiratory Rate: 17 breaths/min

Temperature: 98.4°F

BMI: 28.6 kg/m²

General examination was unremarkable. The patient was conscious, alert, and oriented.

5. Laboratory Investigations

Test	Result	Reference Range
Fasting Blood Glucose	148 mg/dL	70–99 mg/dL
HbA1c	7.2%	Below 5.7%
Total Cholesterol	214 mg/dL	Below 200 mg/dL
LDL Cholesterol	142 mg/dL	Below 100 mg/dL
HDL Cholesterol	42 mg/dL	Above 40 mg/dL
Triglycerides	168 mg/dL	Below 150 mg/dL
Hemoglobin	14.2 g/dL	13–17 g/dL
Serum Creatinine	0.9 mg/dL	0.7–1.3 mg/dL
ALT	32 U/L	7–56 U/L
AST	28 U/L	10–40 U/L

6. Diagnostic Findings

The laboratory findings indicate elevated fasting blood glucose and HbA1c levels.

The HbA1c level of 7.2% is consistent with diabetes mellitus when interpreted in the appropriate clinical context.

The lipid profile also demonstrates elevated LDL cholesterol and triglycerides.

Renal and liver function parameters are within the reported reference ranges.

7. Assessment

Primary Assessment:

1. Type 2 Diabetes Mellitus — newly identified
2. Dyslipidemia
3. Overweight
4. Sedentary lifestyle

Risk Factors Identified:

- Family history of diabetes
- Elevated BMI
- Sedentary lifestyle
- Irregular dietary pattern

- Elevated LDL cholesterol

8. Treatment and Management Plan

The patient was advised to:

1. Follow a balanced diet with controlled carbohydrate intake.
2. Increase physical activity gradually, subject to physician guidance.
3. Monitor blood glucose regularly.
4. Maintain adequate hydration.
5. Reduce consumption of processed foods and foods high in saturated fats.
6. Work toward gradual and sustainable weight reduction.
7. Follow up with the physician for evaluation of glucose and lipid control.

Medication decisions should be made by the treating physician based on the patient's complete clinical assessment.

9. Follow-Up

A follow-up consultation was recommended after approximately 8–12 weeks.

Repeat investigations may include:

- Fasting blood glucose
- HbA1c
- Lipid profile
- Blood pressure assessment
- Weight/BMI assessment

10. Important Note

This document is a **fictional sample created for software development, testing, and RAG application demonstrations**. It is not a real patient's medical record and should not be used for medical diagnosis or treatment.

Physician: Dr. A. Mehta

Department: General Medicine

Hospital: CityCare Multispeciality Hospital